GIG CYMRU NHS WALES
Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

CONTRACTS FOR INNOVATION CYMRU
CONTRACTAU AR GYFER ARLOESI CYMRU

Llywodraeth Cymru
Welsh Government

Contracts for Innovation Cymru

Briefing Event

2nd October 2026 9:30 to 12:00

This event is being recorded

Welcome and Etiquette

This event is being recorded with cameras and microphones disabled.

Questions will be reviewed at the end of each challenge section for the Q&A session

If we can kindly ask you to not use the Chat but the Q&A function only.

Agenda

09:30 **Welcome and overview**

- Welcome and Introduction
- Contracts for Innovation Cymru
- Welsh Government Innovation
- Innovate UK
- SimplyDo

10:00 **Challenge** and **Q&A:** Reducing missed appointments and improving access to planned care

10:30 **Challenge** and **Q&A:** Modernising the bowel screening pathway in Wales

11:00 **Challenge** and **Q&A:** Ending abandonment after stroke

11:30 **Challenge** and **Q&A:** Transforming multi-agency safeguarding for high-risk domestic abuse in south Wales

12:00 *Closure and final Q&A*

CONTRACTS FOR INNOVATION CYMRU
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Working across sectors to identify unmet needs

Supporting public sector to develop challenges, with an emphasis on relevance to Welsh businesses

Work with funding providers to secure funds for challenges, to enable 100% funding for R&D

Full project management, including engaging with stakeholders, management of all documents and governance

Supporting challenge owners and innovative suppliers to work collaboratively.

Provide recognised training at a reduced cost i.e. PRINCE 2.

Llywodraeth Cymru
Welsh Government

www.cymru.gov.uk

Wales Innovates

Contracts for Innovation

Steve Parkes (Collaboration Specialist)

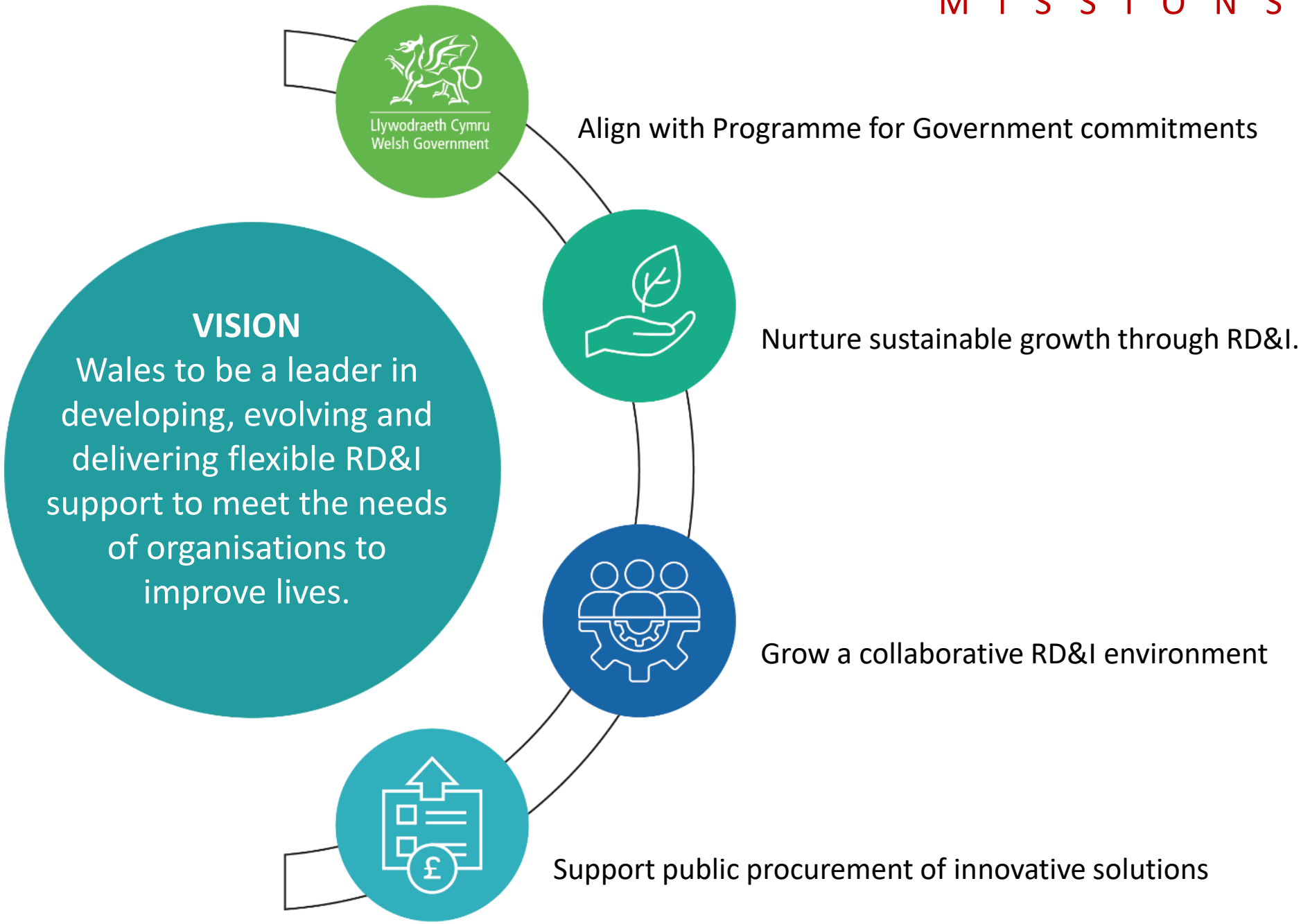
UKRI Innovate UK | Contracts for Innovation

Innovation is –
about developing new ideas into products and services.
It can mean inventing new things.
Or making older things work better



Innovation aims:
Make things better for people
Include everyone
Be flexible
Have people working together.

M I S S I O N S



Align with Programme for Government commitments

Nurture sustainable growth through RD&I.

Grow a collaborative RD&I environment

Support public procurement of innovative solutions

Llywodraeth Cymru
Welsh Government

www.cymru.gov.uk

SMART Flexible Innovation Support (SFIS)

SMART FIS is a unique Welsh Government Scheme which aims to help Welsh organisations achieve "Innovation Excellence".

- Delivered by a team of specialists
- Access to UK & International networks and funding opportunities
- Financial support – grant and **procurement (public sector)**
- Co-fund activities
- Specialist consultancy
- Collaboration

Contracts for Innovation

- Innovate UK Contracts for Innovation (formerly known as the Small Business Research Initiative) helps public sector organisations to solve complex challenges by running a competitive funding opportunity, to develop and adopt new solutions and technologies.

- The Contracts for Innovation funding mechanism is highly flexible. It allows public sector organisations, as the **Challenge Owner**, the ability to shape competitions to fully meet their needs.

Through Contracts for Innovation competitions, public sector organisations can support:

- Early-stage feasibility studies
- Prototype development
- Late-stage demonstrator projects

UKRI Innovate UK | Contracts for Innovation


CircularEconomy
Women
Hydrogen
Football
HealthcareSpaces
FoodSupplyChains
Coalfields
Cancer
Agriculture
sHealth
AnimalWelfare

Welsh Government Contracts for Innovation

Presented on our approach to numerous EU regions, Australia and South Africa

Invested in Contracts for Innovation Cymru (Betsi Cadwaladr University Health Board)



St Davids Award winner – Covid Challenge – Rapid sanitisation of ambulances

https://www.ambulance.wales.nhs.uk/Default.aspx?gcid=1753&lan=en

Government Opportunities excellence winner – Childhood obesity/affordable nutritious food

https://foodmanagement.today/welsh-organisations-takes-steps-to-tackle-childhood-obesity/

Innovate UK | Contracts for Innovation

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Contracts for Innovation Key Features:

Contracts

- 100% funded R&D (procurement contract for R&D Services)
- Payments made on completion of milestones
- Costs quoted must reflect actual costs and not include profit
- Contract with a single organisation - who may choose to sub-contract but remains accountable

IP rests with Supplier

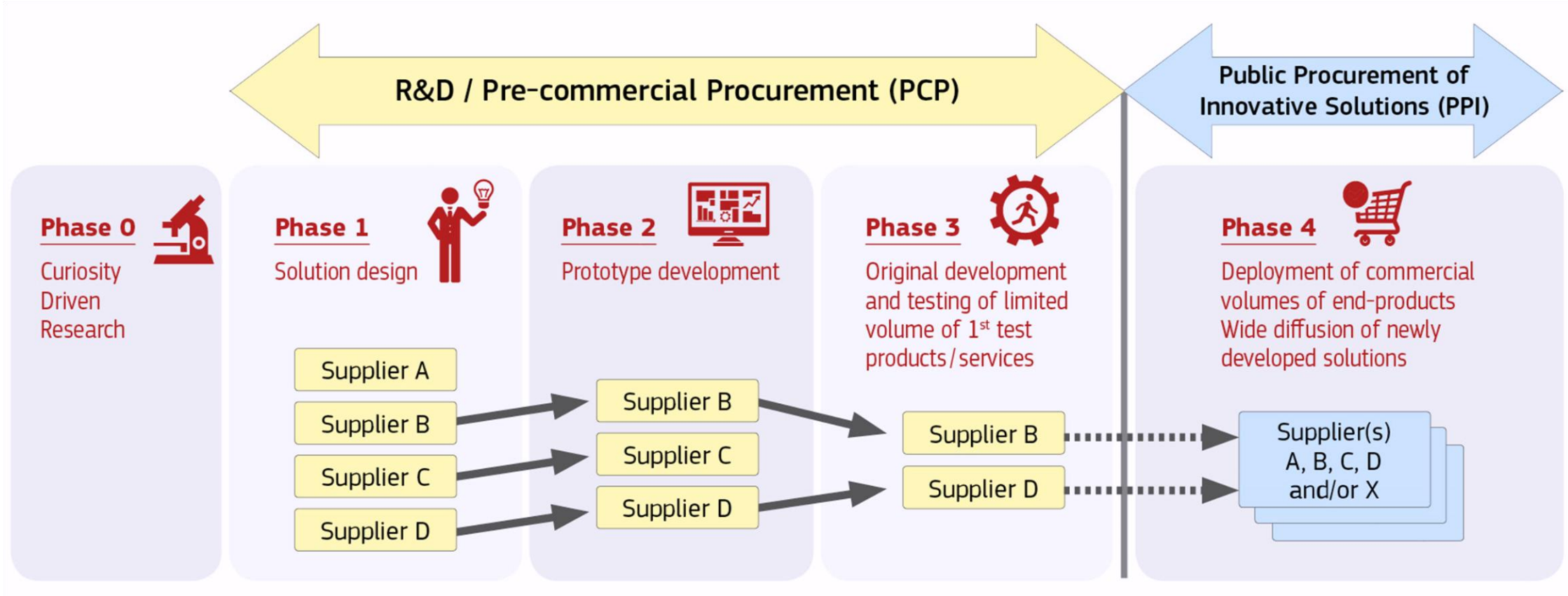
- Certain usage rights for the Public Sector (licenses etc)

Eligibility

- Can be used to adapt technology or processes from other sectors as long activity is at least 50% Research & Development
- Open to all organisations. There is no limit on the size or type of company
- All organisations must demonstrate a route to market

The Process

The below is an example only. Contracts for Innovation can be applied anywhere from TRL 2-8 and can start/conclude anywhere along the process.



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Four challenges are being launched, each with their own application page on Simply Do.

Please carefully consider the in and out of scope sections contained on each challenge page.

Draft contract has been included in the supporting information for each challenge. This is a UK wide template, therefore not subject to change. By submitting your application, you are confirming your acceptance of the terms and conditions.

CONTRACTS FOR INNOVATION CYMRU
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Reducing missed appointments and improving access to planned care

Phase 1 Funding - Feasibility
Funding Available: Up to £200,000

Number of Projects: Current funding is available to support a portfolio of projects, dependent upon the number/quality of submissions received. We are seeking a broad range of projects, from rapid or low-cost demonstrations to larger scale demonstrators reaching a wider demographic.



Transforming multi-agency safeguarding for high-risk domestic abuse in south Wales

Phase 1 Funding - Feasibility

Funding Available: Up to £60k per project exc. VAT
Number of Projects: Up to 6 projects

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Modernising the bowel screening pathway in Wales

Phase 2: Development and Real-World Demonstration

Total Funding Available: £ 300,000
Number of Projects Anticipated: Current funding of £300,000, excluding VAT, is available to fund 4-5 projects which may be subject to change.



Ending abandonment after stroke: how can Wales deliver better recovery beyond hospital?

Phase 2: Development and Real-World Demonstration

Funding Available: £300,000 exc. VAT

Number of Projects: Current funding is available to support a portfolio of projects, dependent upon the number/quality of submissions received. We are seeking a broad range of projects, from rapid or low-cost demonstrations to larger scale demonstrators reaching a wider demographic.

Key Dates:

Challenge launch	22nd September 2026
Close date	12noon, 20th October 2026
Assessment start	21st October 2026
Shortlist agreed & suppliers notified	November 2026
Meet and greet with suppliers	TBC
Decision release	November 2026
Contracts awarded	November 2026
Signed contracts returned & finalised	November 2026
Projects commence	November 2026
Projects complete	March 2027
Final Report submission deadline	March 2027
Final Project Board	March 2027

**** Please note dates may be subject to change****

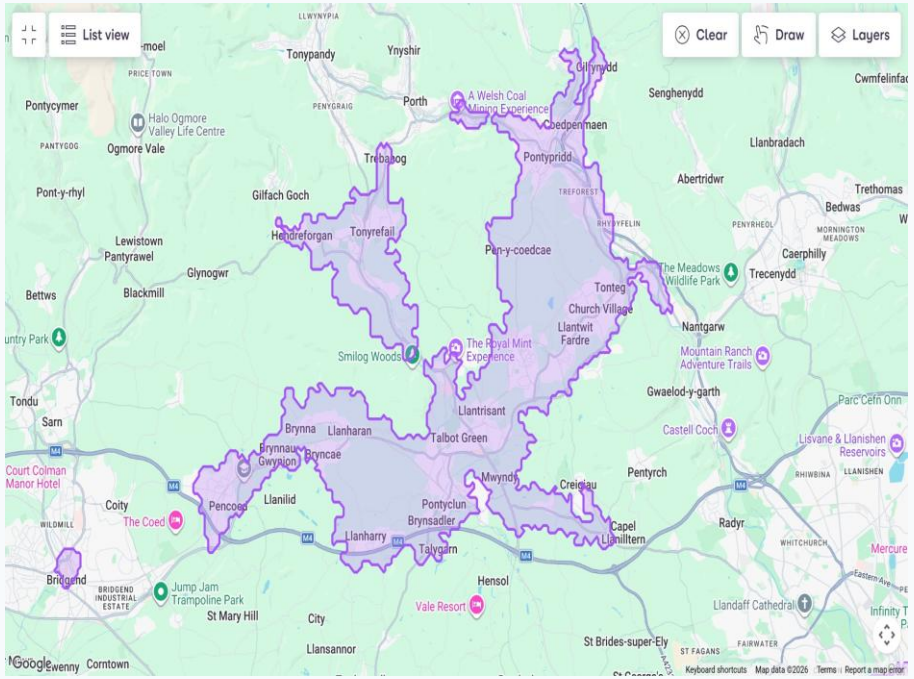
Simply Do

Josh Barnett

REDUCING MISSED APPOINTMENTS

AND IMPROVING ACCESS TO PLANNED CARE

An open innovation challenge to understand and address the communication, practical, behavioural, accessibility and service barriers that affect outpatient attendance.



Cwm Taf Morgannwg University Health Board
In partnership with Contracts for Innovation Cymru

RIC Hub

GIG CYMRU NHS WALES
Bwrdd Iechyd Prifysgol Cwm Taf Morgannwg University Health Board

Supplier briefing | 2 October 2026

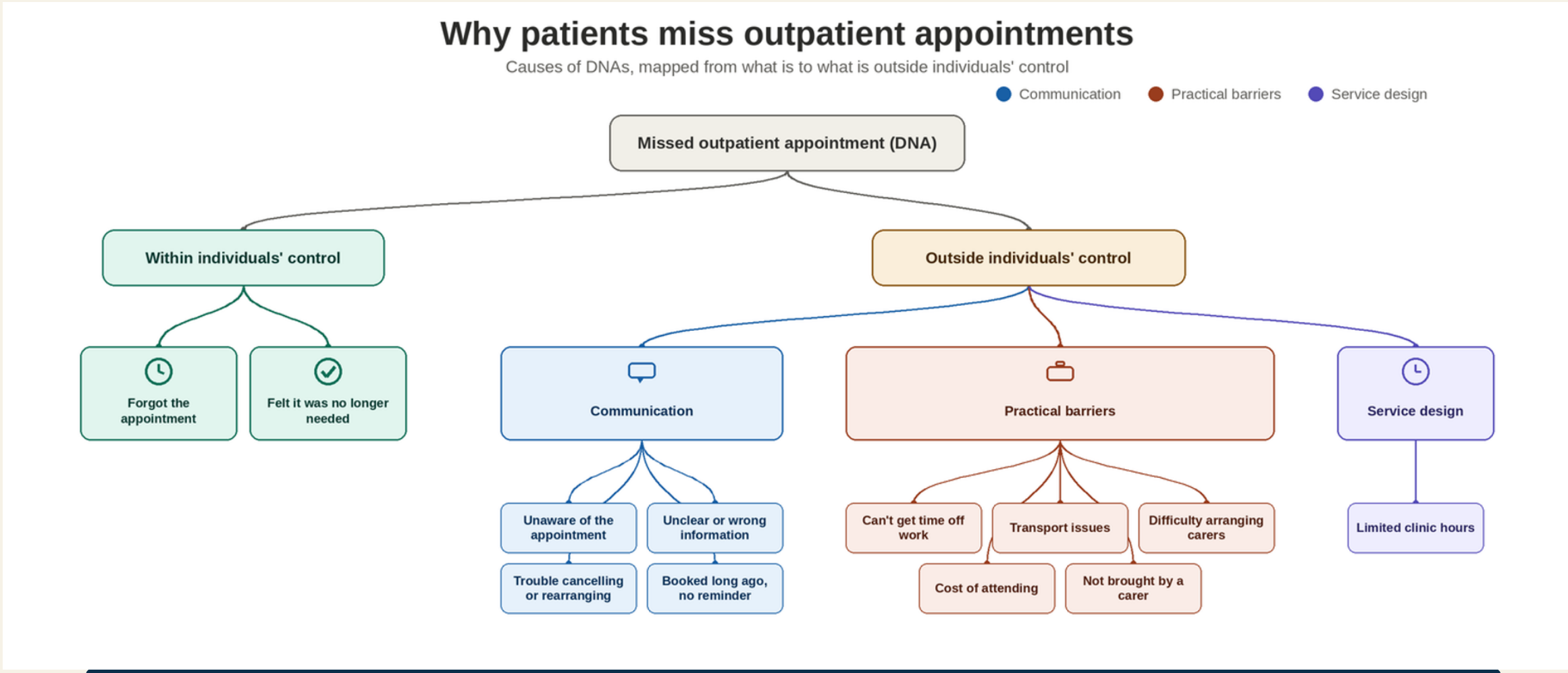
CASE FOR CHANGE

Why this matters now

The figures describe the scale; Phase 1 should refine baselines, variation and causal understanding.

8.6%	64k	£9.6m	82k	21.3%
Average outpatient DNA rate	Approximate missed appointments	Estimated annual lost value	People on outpatient waiting lists	Households without car or van

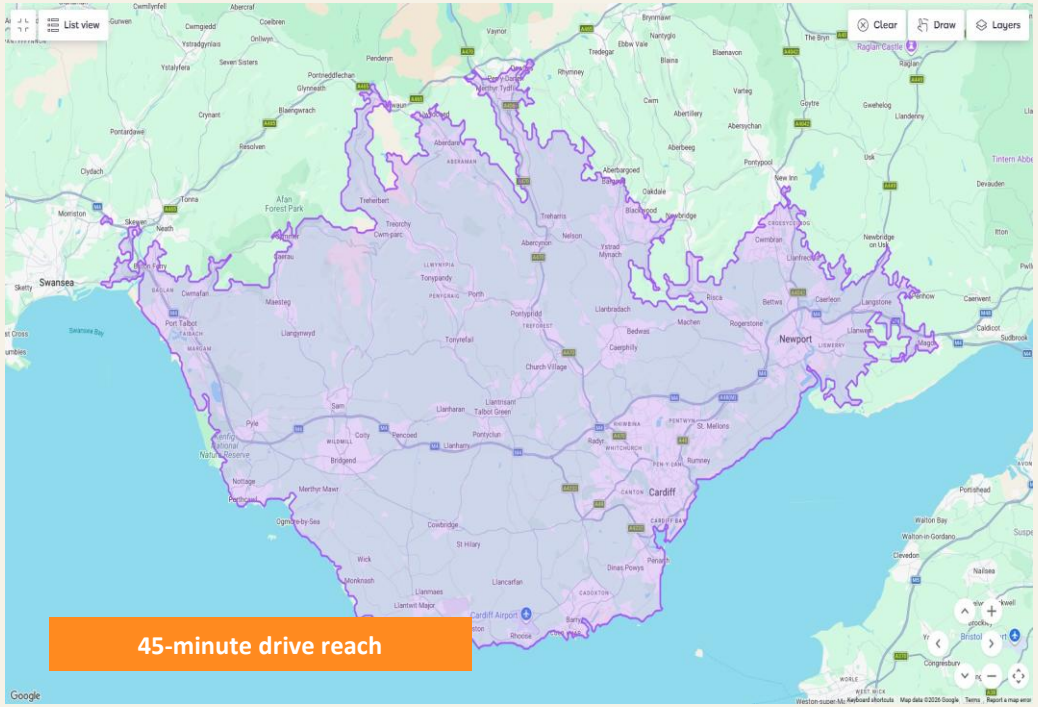
- Each missed appointment is a lost opportunity for care and can delay diagnosis, treatment or monitoring.
- Unused clinic time reduces productivity while demand and waiting lists remain high.
- Headline improvement must be assessed alongside equity, experience and unintended consequences.

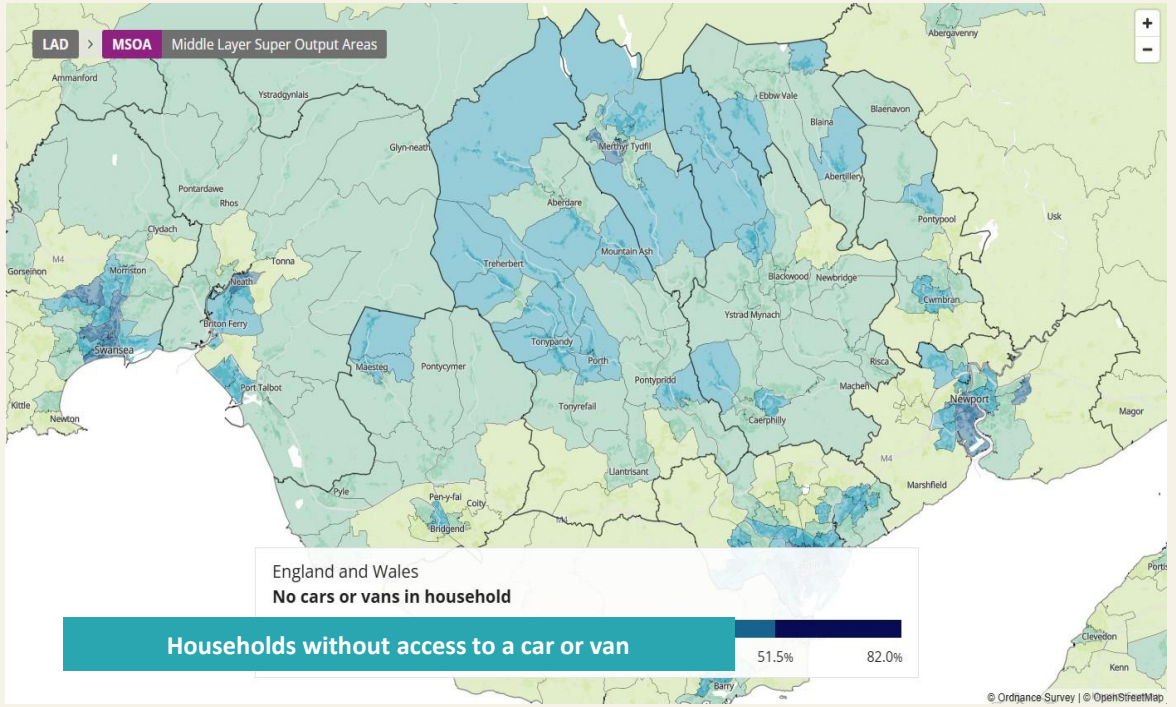


Applicants should show how discovery work will distinguish modifiable barriers from factors outside the intervention’s control.

The same appointment can create very different barriers

Travel time, service location, appointment timing and household circumstances interact.





Transport and accessibility barriers are in scope. Projects focused primarily on providing, commissioning or subsidising transport are outside the intended scope.

EXISTING LANDSCAPE

Build on existing transformation, do not duplicate it

The challenge should complement current patient-contact, outpatient and NHS Wales digital developments.

Existing context

- Patient Centred Contact Transformation
- Evolving outpatient contact and communication arrangements
- NHS Wales App development
- Established NHS Wales patient administration infrastructure

What this challenge adds

- Understanding why people miss appointments
- Inclusive patient and staff co-design
- Targeted discovery across populations and settings
- Feasibility testing and evidence for adoption

Supplier implication

- Explain how the proposal complements existing capabilities
- Avoid assuming a blank digital landscape
- Identify dependencies early
- Design for both digital and non-digital access

Phase 1 is a test-and-learn route before any decision about wider deployment or adoption.

DIGITAL ENVIRONMENT

Interoperability: proportionate to the Phase 1 proposal

Applicants need to distinguish what must be integrated now from what must be capable of integration later.

1 **Prototype safely**

Use fictional, synthetic or appropriately governed data where suitable for early design and testing.

2 **Describe dependencies**

Specify systems, data fields, APIs, workflows and technical support genuinely required in Phase 1.

3 **Plan future fit**

Explain how the concept could integrate with NHS Wales administrative and clinical systems if it progresses.

Working assumption: live integration is not a universal Phase 1 requirement; requirements will depend on the proposed solution and be agreed during mobilisation.

PHASE 1

Discover, design, test and learn

Phase 1 should generate credible learning, not begin with full enterprise deployment.

1

Discover

Analyse patterns, map workflows and understand variation by service, place and population.

2

Design

Co-design inclusive approaches with patients, booking teams, clinicians and partners.

3

Test

Develop prototypes and undertake proportionate usability or feasibility testing.

4

Learn

Assess acceptability, feasibility, safety, equity, value, scalability and future integration.

The strongest proposals will connect an explicit theory of change to measurable, equitable outcomes.

INCLUSIVE INNOVATION

Who should shape the solution?

Recruitment and access arrangements will be agreed during mobilisation, subject to capacity, consent and governance.

- Booking and administrative staff
- Relevant clinicians and service managers
- Patients with planned-care experience
- Welsh speakers
- People with limited digital access or confidence
- Disabled people and those with communication needs
- People facing transport, work, caring or financial barriers
- Communities experiencing poorer access or higher deprivation

Design for varied levels of digital access, health literacy, language need and accessibility from the outset.

EVIDENCE AND VALUE

How we will recognise useful learning

Success is broader than a single aggregate DNA percentage.

Patients and communities

- Attendance and cancellation outcomes
- Successful appointment communication
- Experience and ease of access
- Differential effects by population group and geography

Staff and services

- Clinic utilisation
- Booking and administrative workload
- Workflow fit and usability
- Waiting-time or capacity implications

System and adoption

- Cost, value and sustainability
- Safety and unintended consequences
- Scalability and affordability
- Future integration and adoption requirements

Application prompt: What change will your approach produce, for whom, through what mechanism, and how will you know?

ASSURANCE

Governance, approvals and clinical safety

Requirements will be proportionate to the proposed activities, data use and risk profile.

Evaluation and research

- CfIC will determine the appropriate evaluation or research route.
- Applicants should clearly describe participants, methods, data use and intended generalisability.

Information governance

- Data minimisation, privacy, transparency and secure handling are expected from the start.
- A relevant IG route will be identified during mobilisation.

Digital assurance

- Cybersecurity and technical assurance will reflect the maturity, intended use and risk profile of the proposed approach.

Clinical safety

- Suppliers proposing solutions should identify applicable clinical-safety duties, hazards, documentation and responsibilities.

Items still to be confirmed through the formal competition route: approval timescales, local Clinical Safety Officer arrangements and the named information-governance contact.

PARTNERSHIP OFFER

What CTM anticipates facilitating

Support will be proportionate and subject to project needs, capacity and relevant approvals.

CTM anticipates facilitating

- Access to relevant operational and clinical stakeholders
- Engagement with booking and administrative teams
- Appropriate patient and public involvement routes
- Appropriately anonymised or aggregated attendance information
- Digital, analytics, IG and clinical-safety discussions
- Understanding of current workflows and feasible test environments

Applicants should specify

- People, services and staff time required
- Data fields and level of detail required
- Technical dependencies and test environment
- Patient recruitment requirements
- Approval and safety documentation
- What remains deliverable if some dependencies are unavailable

Be specific early: clear dependencies make a Phase 1 proposal easier to assess, mobilise and govern.

CLOSING

What a strong application will show

Innovation is valuable when it addresses a real barrier, can be tested safely and produces credible evidence.

1. **A credible understanding of the access problem**
2. **A feasible Phase 1, not a full-scale deployment**
3. **Meaningful patient and staff co-design**
4. **Welsh language, accessibility and digital inclusion**
5. **Clear outcomes and an evaluation approach**
6. **Transparent data, integration, governance and safety needs**
7. **Scalability, affordability and potential adoption**
8. **Complementarity with existing transformation**

We are looking for partners who can understand the problem precisely, work with patients and staff, test promising approaches safely, and show what could improve attendance and equitable access.

Diolch yn fawr pawb

Thank you everyone

Any questions ?



Application portal links:



Reducing missed appointments and improving access to planned care



Transforming multi-agency safeguarding for high-risk domestic abuse in south Wales



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ContractsforInnovationCymru@wales.nhs.uk

Contracts for Innovation Cymru Website

Applying for Contracts for Innovation Cymru Challenges - guidance and tips

Applying for Challenges - guidance and tips (youtube.com)